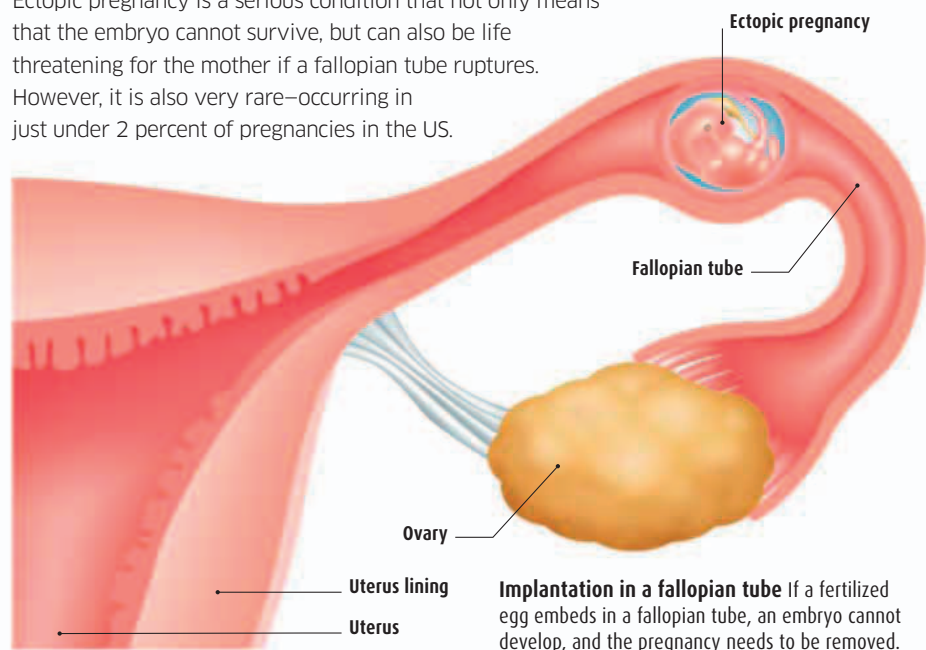


Q What is an ectopic pregnancy?

This is a pregnancy in which a fertilized egg implants outside the uterus. In about 95 percent of cases, ectopic pregnancy occurs in a fallopian tube, but it can also happen, for example, in the cervix, on an ovary, or in the abdominal wall.

Ectopic pregnancy is a serious condition that not only means that the embryo cannot survive, but can also be life threatening for the mother if a fallopian tube ruptures. However, it is also very rare—occurring in just under 2 percent of pregnancies in the US.



Q How will I know if I have an ectopic pregnancy?

There is no one set of symptoms for an ectopic pregnancy. Some women may have no signs or symptoms at first, or the body may respond to the implantation of the egg as a normal pregnancy and respond accordingly. In this case, it can take up to 10 weeks for the symptoms of ectopic pregnancy to appear. Only when the ectopic pregnancy is discovered at a routine ultrasound, or the blastocyst gets so big as to cause swelling or rupture at the implantation site will you receive warning signs that something is wrong. Early symptoms can include:

- » Severe cramping, similar to bad period pain.
- » Pain and tenderness on one side of the abdomen.
- » Vaginal bleeding.
- » Stomach upset, such as diarrhea and nausea.
- » Pain on defecation.
- » Shoulder-tip pain.

Q Are there circumstances that increase my risk of an ectopic pregnancy?

You are at increased risk of ectopic pregnancy if you have had previous problems with your fallopian tubes, such as damage caused by an infection such as chlamydia, or a previous ectopic pregnancy; IVF treatment; patches of endometriosis that have spread to the fallopian tubes; pelvic inflammatory disease; or an appendectomy or other abdominal surgery that left scars. Some types of contraception, the IUD and progesterone-only pill, also increase the risk of ectopic pregnancy since they work by preventing implantation in the uterus, but they do not always prevent fertilization.

Q How will my ectopic pregnancy be dealt with?

An ectopic pregnancy is a medical emergency and you will need to go to the hospital. Once there, you will be given an internal ultrasound, during which a long, thin ultrasound scanner is inserted into your vagina, to check whether there are any signs of pregnancy within your uterus. A blood sample will be taken to measure your levels of the pregnancy hormone human chorionic gonadotrophin (hCG). Depending on the level of the hormone and your symptoms, one of the following will be recommended:

» **Expectant management** means that your ectopic pregnancy will be expected to resolve itself naturally without intervention. You will be closely monitored in the hospital.

» **Medical management** will be suggested if your pregnancy is in its very early stages. You will be given an injection to prevent the fertilized egg from growing any further. It will die and then be reabsorbed into your body's tissue.

» **Surgical management** will be given if there is a high chance that you will suffer a rupture in your fallopian tube. Surgery will be performed under general anesthesia to remove the fertilized egg and possibly to remove the affected tube. Although sometimes the tube can be saved, losing one tube doesn't affect your chances of becoming pregnant again. Unless your tube has already ruptured and you are not well enough to give consent, you will always be asked before you go into surgery if you agree to the affected tube being removed if necessary.

Q Will I be able to have a normal pregnancy after an ectopic one?

Yes. Although there's a slightly increased chance of suffering another ectopic pregnancy, statistically you are far more likely to go on to have a safe and healthy pregnancy next time.

65% of women have a **successful pregnancy within 18 months** of an ectopic pregnancy. It's thought that this figure rises to **85 percent after two years.**